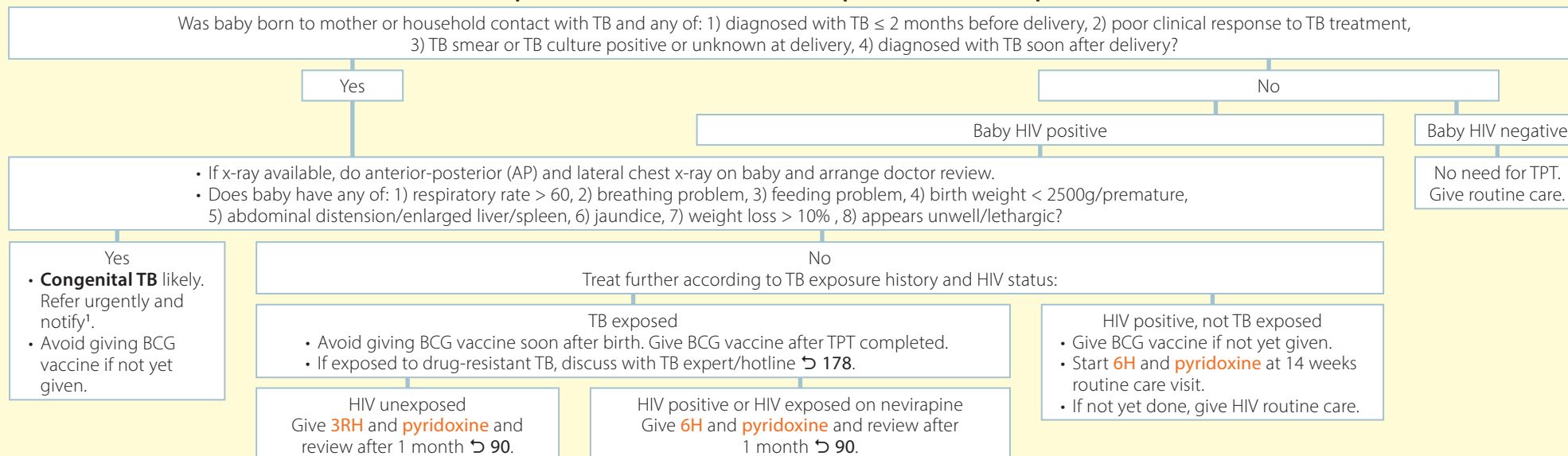


PREVENT COMMUNICABLE INFECTIONS IN THE NEWBORN

Assess and manage the newborn exposed to HIV, TB, hepatitis B or syphilis. If exposed to HIV, assess and manage further ➔ 168.

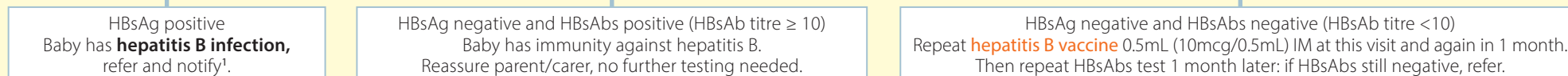
Assess the newborn exposed to TB, or the newborn who tests positive for HIV, for TB preventive treatment (TPT)



Manage the baby born to mother with hepatitis B infection

- Arrange delivery at a facility that stocks immunoglobulin (HBIG) and the monovalent hepatitis B vaccine:
 - Give **hepatitis B immunoglobulin (HBIG)** 200IU IM and **hepatitis B vaccine** 0.5mL (10mcg/0.5mL) IM, as soon after delivery as possible, within 12-24 hours.
- Continue routine hepatitis B immunisations at 6, 10, 14 weeks and 18 months.

Arrange follow up when baby is 9 months old: take blood from baby for HBsAg and hepatitis B surface antibodies (HBsAbs):



Manage the baby born to mother with syphilis

- If rash (peeling rash, red/blue spots or bruising especially on soles and palms), jaundice, pallor (pale conjunctiva/palms of hands), distended abdomen, swelling, birth weight < 2500g, runny nose, respiratory distress, hypoglycaemia, **congenital syphilis** likely. Refer urgently and notify¹.
- If no signs/symptoms of congenital syphilis and any of the following, give baby single dose **benzathine benzylpenicillin** 50 000 units/kg IM into outer thigh, and discuss/refer:
 - 1) Mother received < 3 doses of benzathine benzylpenicillin injections
 - 2) Mother received antibiotic other than benzathine benzylpenicillin to treat syphilis
 - 3) Delay (> 14 days) between maternal doses of benzathine benzylpenicillin
 - 4) Baby delivered within 30 days of mother receiving last dose of benzathine benzylpenicillin

¹Complete notifiable medical conditions (NMC) case notification form and send to NMCsurveillanceReport@nicd.ac.za or notify electronically: <https://www.nicd.ac.za/nmc-overview/notification-process>.